

Management Response

Evaluation Title	Mid-term Evaluation of the Global Task Force on Cholera Control (GTFCC)
Commissioning Unit	WHO Evaluation Office
URL Link to the evaluation	Report, Annexes, Brief
WHO Biennial Evaluation Workplan	WHO Executive Board-approved Organization-wide Evaluation Workplan for 2024-2025
Unit Responsible for providing the management response	HQ/WHE/EPM/EZH
Overall Management Response: Accepted Preamble: The Global Task Force on Cholera Control (GTFCC) is a network of more than 50 partner institutions, including governmental and non-governmental organizations, UN agencies, and scientific partners, all working towards global cholera control and prevention. The World Health Organization (WHO) hosts the GTFCC Secretariat (the Task Force’s coordinating entity), in accordance with the mandate given to WHO by the Members States through World Health Assembly resolutions 44.6, 64.15 and 71.4, to establish the GTFCC and coordinate the implementation of the Global Roadmap to end cholera by 2030, launched in 2017. The overall objective of the strategy is to achieve a 90% reduction in cholera deaths by 2030 and to eliminate cholera in as many as 20 countries. This will be achieved through multi-sectoral interventions to (i) prevent cholera through the implementation of a set of measures - such as long-term WASH - in areas most affected by cholera (Priority Areas for Multisectoral Interventions – PAMIs) and (ii) by containing outbreaks through early detection and rapid response to alerts, supported by (iii) a robust and effective coordination mechanism. The purpose of the GTFCC is to support increased implementation of evidence-based strategies to control cholera. The GTFCC aims to achieve this through strengthened international collaboration and improved coordination amongst stakeholders active in cholera-related activities. GTFCC activities will aim to raise the visibility of cholera as a public health issue, facilitate sharing of evidence-based practices, and contribute to capacity development in all areas of cholera control. In 2024, the GTFCC Steering Committee and Secretariat commissioned an independent mid-term evaluation to assess progress in the implementation of the Global Roadmap to end cholera by 2030, identify challenges and lessons learned, and inform the strategic direction of the partnership toward 2030. General comments: WHO and the Global Task Force on Cholera Control (GTFCC) welcome the mid-term evaluation of the Task Force, that included an assessment of the design and implementation of both the GTFCC as a platform as well as the operationalization of the Global Roadmap Strategy (2017-2030), <i>Ending Cholera – A Global Roadmap to 2030</i>. The evaluation provides useful learning that will inform the strategic and operational future of the GTFCC, help identify ways to better adapt and effectively deliver the 2030 Roadmap, as well as to enhance planning, reorientation and reprioritising as needed, implementation and programme performance – all of which are required to ensure successful adaptation to the rapidly changing global landscape marked by significant epidemiological, political and climatic changes, and risk profile for cholera. Implementation will be prioritised based on available resources, with a focus on high-burden countries and interventions with the greatest public health impact, including identifying and targeting priority areas for multisectoral interventions (PAMIs) The GTFCC operates as a multi-stakeholder partnership platform. In this context, WHO plays a dual role in line with its mandate provided by Member States through the WHA. First, it serves as the host and coordinating entity of the GTFCC Secretariat, facilitating collaboration and alignment across the partnership. Second, WHO contributes directly to cholera prevention and control through its technical programmes at global, regional and country levels. Within this arrangement, WHO facilitates coordination, convening, and technical alignment across the partnership, while individual GTFCC members implement activities under their respective institutional mandates, governance structures, and accountabilities. WHO also retains leadership for normative guidance and technical implementation through its global, regional and country programmes, ensuring alignment	

between GTFCC coordination and operational delivery. This governance model enables the GTFCC to leverage the comparative advantages of its partners while maintaining WHO's stewardship role in coordinating global efforts to support countries in cholera prevention and control.

Following from the initial review of the GTFCC in 2017 and this evaluation that covered the period 2017 to May 2024, the findings and recommendations will support further clarifying the functioning, roles and responsibilities across the Task Force and its members with a particular focus on coordination and programmatic delivery at the global, regional, and country levels.

Given the highly constrained environment and reduced resources - including limited human resources - it will be critical to clearly identify the resources available to implement the proposed actions outlined below.

Management Response Status	<i>In Progress</i>
Date	April 2026

Recommendations and Action Plan

Recommendation 1: To effectively implement the Roadmap through 2030, develop a strategic action plan with prioritised objectives, a results framework, costed work plan, budget and clearly defined stakeholder roles.

Sub-recommendations:

- i. Review priority objectives to 2030 (as well as activities and outcomes), ensuring an appropriate balance across outbreak response and prevention, OCV and WASH, and integration with other disease/ epidemic control efforts
- ii. Develop a results framework, including specifying partners' contributions
- iii. Develop a prioritised costed workplan to 2030, taking into account priorities and prospective resource availability.

Management response	Accepted. Review GTFCC priorities in light of the 2030 targets and develop an inclusive, country-driven and partner-driven costed workplan.			
Status	<i>In Progress</i>			
Key actions	<i>Responsible Entity(ies) – Lead entity in bold</i>	<i>Timeline</i>	<i>Status</i>	<i>Comments</i>
Review and validate whether the GTFCC's current priorities remain appropriate in the evolving environment and under increasing constraints.	WHE/EPM–GTFCC Secretariat, GTFCC SC, GTFCC CSP, HEP/ECH/WSH, WHE/CRS/HCR	30 March 2026	Completed	This was done through a structured consultation and prioritization exercise with Steering Committee members to review guiding principles and priorities to 2030 in the context of the global crisis and constrained environment. Based on this, determine whether the Roadmap Theory of Change requires refinement or renewed commitment. It includes discussing and clarifying responsibilities between GTFCC and other existing mechanisms (e.g., GOARN, partners in their respective individual capacity) with regards to coordinating outbreak response or leveraging the GTFCC network to support outbreak response versus preparedness activities (reference to the three Axes of the Roadmap, role of the GTFCC and articulation between those axes and other mechanisms).

Using the results of Step 1, develop a prioritized inclusive, country-driven and partner driven costed Workplan	WHE/EPM – GTFCC Secretariat , GTFCC SC, with ECH/WSH, BOS/PRP	30 June 2026	In Progress	The development of a prioritized inclusive, country-driven and partner-driven workplan will guide and coordinate GTFCC efforts toward achieving the Roadmap’s 2030 objectives over the remaining four years. This plan will include an updated results framework—with prioritized objectives, clarified member/partner roles—and a costed workplan. In developing the results framework, attention will be given to alignment with the WHO GPW14, strengthening the global architecture for health emergency prevention, preparedness, response (HEPR) and resilience framework for WHO Health Emergencies and WHO related strategies and guidance related to cholera (including multi-sectoral interventions) strategies of other UN organizations and implementing partner strategies and contributions will be also aligned. This exercise will consider reorganizations in WHO (and other UN organizations/partners), and the current financial constraints WHO, partners and countries face.
Develop an evidence-based country categorization framework for cholera to guide differentiated technical support, prioritize actions, and accelerate operationalization of the Roadmap toward the 2030 milestone.	WHE/EPM – GTFCC Secretariat, GTFCC EPI TWG with other TWGs	31 December2026	In progress	Roadmap target countries have expanded from 47 in 2017 to 52 in 2023, highlighting the need for an adaptive framework that responds to changing epidemiological realities. The 2024 GTFCC Mid-Term Review found mixed progress, including slower-than-expected declines in mortality and an increasing number of countries with active outbreaks. Developing a country categorization framework that relies on a data-driven approach - using epidemiological evidence as the primary classification criteria, building on existing surveillance guidelines and complemented by cholera vulnerabilities and operational factors - will refine the Roadmap’s implementation strategy. It will create structured progression pathways to guide countries toward ending cholera based on their current epidemiological situation. This framework will also address several key findings of the Mid-Term Review, including the need to prioritize actions to operationalize the Roadmap (Finding 1.2), strengthen coordination and alignment at regional and country levels (Finding 2.2), establish a monitoring and evaluation framework to track differentiated results (Finding 4.1), and support advocacy and resource mobilization (Finding 4.7).

Recommendation 2: Enhance engagement of GTFCC partners at country and regional levels to maximise results at country level.

Sub-recommendations:

- i. Increase focus on, and priority for, country level work building on progress in developing country NCPs. Identify barriers and address implementation challenges through more specific approaches and greater integration with other disease/ epidemic control and health systems strengthening efforts. Continue to engage with countries to identify WG priorities and increase dissemination and use of WG products among countries
- ii. Strengthen and build on the CSP approach, by 1) identifying and sourcing funding to capacitate the CSP and 2) clarifying the scope and role of the CSP.
- iii. Explore regional approaches to facilitate greater coverage of countries and strengthen cross-border coordination for cholera responses. Enhance engagement with regional partners, including GTFCC members and networks, as well as regional meetings and South to South exchange and learning.

Management response	<i>Accepted.</i> Noting the broad consensus of the importance of supporting countries in implementing their NCPs, implementation of actions by WHO and GTFCC partners will hinge on resource availability, considering the constrained financial environment WHO, countries and partners are facing.			
Status	<i>In Progress</i>			
Design efficient tailored regional approaches for cholera response and cross-regional learning by leveraging existing mechanisms and initiatives.	WHE/EPM – GTFCC Secretariat and TWGs, GTFCC SC, WHO regional offices, GTFCC regional actors, GTFCC CSP, ECH/WSH	31 December 2026	In progress	Design and implement tailored regional approaches for cholera response and cross-regional learning, with regional actors—WHO regional offices, GTFCC regional partners, and public health regional institutions—leading and shaping this work, in collaboration with the GTFCC Secretariat, SC, and CSP. This includes identifying the comparative advantages of existing initiatives and actors, and defining a collaboration plan under the GTFCC umbrella. This work will build on existing tools and mechanisms to maximize current opportunities and efficiencies within the constrained operating environment. For example, the GTFCC has begun working closely with the African Continental Task Force and IMST, with focal points and the Secretariat actively engaged in shaping these future initiatives as part of the broader GTFCC regional agenda. A proposed first milestone is focus on the collaboration with the African continent and an expansion from Q2 2026 to other regions. This exercise will also build on the GTFCC CSP’s ongoing operationalization of a regional hub structure—currently four hubs (Asia, Central Africa, Lake Chad, and Southern Africa)—with National Implementation Officers deployed in priority countries and cross-border coordination mechanisms under development. The CSP’s Phase 2 strategic vision (2025–2030) guides this expansion.
Complementary to Action 1 of Recommendation 2, develop an action plan to optimize country-level support for cholera control, building on midterm evaluation findings.	WHE/EPM – GTFCC Secretariat and FPs, GTFCC SC, GTFCC CSP	30 June 2026	In progress	Building on the initial review and findings of the Evaluation, review success factors and remaining gaps; build on the CSP scope of engagement across its supported countries; map the direct support provided to countries by WHO technical teams and GTFCC partners - both via TWGs and partner individual contributions; identify complementary models for countries without CSP presence; and review mechanisms and resources that support

				countries in developing and implementing national cholera control and prevention strategies, including the Independent Review Panel.
Broaden country representation in the SC and other relevant GTFCC bodies.	WHE/EPM – GTFCC Secretariat, GTFCC SC, GTFCC TWG, GTFCC CSP	30 May 2026	In progress	Fill the third country representative position on the Steering Committee, which is currently vacant. While country representation is already ensured across all relevant technical working groups, expand participation as needed to ensure balanced representation from all cholera-affected regions.

Recommendation 3: Clarify roles and responsibilities of GTFCC core structures to improve partner engagement, ownership and facilitate decision making.

Sub-recommendations:

- i. Steering Committee: 1) clarify its decision-making role in line with WHO hosting approaches and rules, and clarify expectations on strategic direction and oversight; 2) Consider expanding and diversifying its composition (notably from WASH and country stakeholders), without making it too large.
- ii. Secretariat: in addition to overall coordination of GTFCC, 1) reinforce its role in relation to implementation of the Roadmap strategic action plan (see Recommendation 1), with partners taking on additional roles and responsibilities in areas where they have specific capacity/comparative advantage; and 2) clarify its role vis a vis the WHO Cholera Program and dedicate an FTE for the GTFCC Secretariat.
- iii. Working Groups: 1) strengthen systematic cross-WG coordination of priorities, work plans and exchange, 2) consider the need for technical subcommittees (or equivalent) to further specific areas building on members' motivation and available resources, and 3) reassess the need to operationalize the Risk, Communication, and Community Engagement (RCCE) WG.
- iv. The IRP: Assess continuing need for the IRP in light of challenges met and limited availability of resources and/ or measures to improve timeliness of IRP support.
- v. General Assembly: Expand partner engagement within the GTFCC by increasing 1) the contribution and role of WASH and development partners and 2) participation of countries, for example by holding some General Assembly meetings in cholera-affected countries and 3) involvement of multiple sectors.

Management response	<i>Accepted.</i>			
Status	<i>In Progress</i>			
Key actions	<i>Responsible Entity(ies)</i>	<i>Timeline</i>	<i>Status</i>	<i>Comments</i>
Conduct a review of, and update as needed, the TORs, accountabilities and working practices of the GTFCC and its key operational structures.	WHE/EPM– GTFCC Secretariat and FPs, with GTFCC SC, GTFCC CSP, DGO/LEG	30 June 2026	In progress	Considering WHO's reorganization, financial constraints, and country needs, GTFCC collaboration and ways of working must be strengthened to ensure effective delivery. The review has started to assess the roles, responsibilities, and purpose of each GTFCC component (incl. Advocacy Task Team), including possible alternatives within WHO rules and procedures, with the aim of enhancing country ownership, sustaining regional leadership and accelerating implementation, while maintaining the structure's current agility.
Conduct 2026-2027 operational planning, including required resourcing for the GTFCC action plan for 2026-2030	WHE/EPM- GTFCC	31 January 2026	Completed	As part of the 2026–2027 WHO operational planning exercise, plan activities and identify the resources required for implementing the GTFCC action plan during this period, as well as any existing gaps.

	Secretariat, PRP			
--	----------------------------	--	--	--

Recommendation 4: Enhance communication, advocacy and resource mobilisation for cholera at the global, regional and country levels to support Roadmap implementation, GTFCC structures, and multi sectoral integrated approaches.

Sub-recommendations:

- i. Develop a communication and advocacy plan based on the strategic action plan (see Recommendation 1), to raise the profile of cholera and identify new opportunities in connection with 1) efforts to reduce stigma of cholera to facilitate more timely and transparent sharing of data by countries, 2) issues of climate change, WASH, and pandemic preparedness and response and 3) by leveraging partnerships' platforms;
- ii. Develop a resource mobilization strategy identifying key priorities linked to the operational plan (see Recommendation 1), targeting high-profile international efforts and positioning cholera in integrated approaches and joint resource mobilization efforts in connection with health and climate change and WASH/ development.
- iii. Explore innovative resourcing strategies, including the use of models to mobilise small grants for local partners to advocate at country level¹ and non-traditional approaches for partner support at global country and regional levels (e.g. secondments, other in-kind support, leveraging partners communication and/ or resource mobilisation teams).

Management response	<i>Accepted</i>			
Status	<i>Not Initiated</i>			
Key actions	<i>Responsible Entity(ies)</i>	<i>Timeline</i>	<i>Status</i>	<i>Comments</i>
Develop an integrated communication and advocacy plan for cholera control and multisectoral action	WHE/EPM – GTFCC Secretariat, DCO, GTFCC SC, GTFCC CSP, GTFCC ATT , ECH/WSH, Comms teams from GTFCC partner institution	30 September 2026	In Progress	This action plan includes assessing required resources, clarifying partner roles at global, regional and national levels, and responsibilities, and identifying the need to revitalize the Advocacy Task Team (and roles at the three levels – Link with Recommendation 2). The plan is currently in early stages and will be developed in collaboration with communications teams at HQ and regional levels, as well as with the SC and key Advocacy Task Team partners. It will also be articulated with the broader WASH agenda and ongoing communication and advocacy initiatives. The plan will contribute supporting countries to maximize sustainable domestic income on climate change, WASH and preparedness investments, and act as a leverage on potential international donors emphasizing an economic positive return on investment.
Map and prioritize resource mobilization targets and needs for the	WHE/EPM – GTFCC	30 September 2026	In progress	The first step is to identify and differentiate targets and needs at global, regional, and country levels, clarify which areas should be prioritized, and

¹ Unitaid and FIND funded small grants to support COVID-19 related advocacy which had a substantial reach across multiple countries and leveraged local capacities and networks. <https://unitaid.org/news-blog/find-and-unitaid-invest-us2-million-to-support-advocacy-for-covid-19-test-and-treat-approaches-in-low-and-middle-income-countries/#en>

WHO GTFCC Secretariat, GTFCC CSP, and the broader GTFCC partnership.	Secretariat, GTFCC SC, GTFCC CSP, EXT/CRM, RM teams from GTFCC partner institutions			outline responsibilities. Based on these needs, develop an updated stakeholder the relevant communication and advocacy strategy to support resource mobilization. This mapping will provide the foundation for a focused and actionable integrated resource mobilization strategy, while also reviewing current funding mechanisms and identifying opportunities for improvement.
--	--	--	--	---

Recommendation 5: Increase engagement, integration and alignment with WASH interventions and programmes highlighting priority WASH areas in the Roadmap and cholera integration in WASH investments at national and subnational levels.

Sub-recommendations:

- i. Strengthen the WASH WG, including by expanding its membership to partners who are familiar with, and can influence, policy making in cholera-affected countries and at the global level (e.g., World Bank, AfDB, UN Water etc.), and by integrating into the WG workplan. Where possible, engage WASH in other WGs.
- ii. Adopt a more holistic/ integrated approach to WASH (relevant to multiple disease control efforts), increase linkages with WASH activities at country, regional and global level and support the transition from ‘emergency’ WASH to more of a long-term WASH focus by strengthening engagement with other organizations’ WASH frameworks, and with relevant events/ initiatives (e.g. UN System-wide Strategy for Water and Sanitation)

Management response	<i>Accepted</i>			
Status	<i>In Progress</i>			
Key actions	<i>Responsible Entity(ies)</i>	<i>Timeline</i>	<i>Status</i>	<i>Comments</i>
Expand the WASH GTFCC Technical Working Group membership to include WASH actors with the capacity to influence policy and decision-making in cholera-affected countries.	WHE/EPM, GTFCC Secretariat and WASH TWG	30 June 2026	In progress	Since 2022, the TWG membership has been broadened to include actors such as the World Bank Group as official members. The WHO WASH programme has an active role in the working group. The expansion to include additional actors with the capacity to influence policy and decision-making in cholera-affected countries is an ongoing process and is expected to continue throughout 2026.
Develop and implement an engagement plan to strengthen collaboration with key stakeholders, including government actors, donors, development partners, and regional bodies.	WHE/EPM, GTFCC Secretariat, GTFCC SC, GTFCC WASH TWG , GTFCC CSP, ECH/WSH, WHE/CRS/HCR	30 September 2026	Not initiated	The engagement strategy will consistently emphasize the rationale for targeting WASH investments in cholera-priority areas, leveraging PAMI analyses to better guide decision-making and align cholera control priorities with broader development-oriented WASH funding opportunities, thereby demonstrating how focused cholera interventions contribute to sustainable WASH outcomes. The engagement plan will build on the recent WASH evaluation, the updated WASH TWG workplan, and partners’ advocacy documents, assessments, and resources (e.g., the WaterAid-led WASH advocacy document for the GTFCC, the World Bank Group consultation on cholera, etc.). Cholera provides a critical lens for identifying gaps in WASH systems, and aligning cholera control efforts within the broader WASH

				investment and strategy landscape strengthens the case for targeted, evidence-based action. There is a need for government-led sector-wide systems to monitor and report on WASH investments in cholera-priority areas at sub-national levels This perspective underscores how strategic WASH improvements directly contribute to reducing cholera transmission, while also reinforcing longer-term system resilience. Efforts must ensure a continuum between short-term reactive operations - such as OCV campaigns or emergency WASH interventions - and longer-term WASH investments, aligned with policies to be defined. Continuous revision and promotion of WASH standards, guidance, and tools, alongside capacity-building and training approaches, should remain a focus at the country level. Strategies for implementation, including training and capacity building at regional and national levels, will be developed in collaboration with WASH TWG partners.
--	--	--	--	---

Recommendation 6: Reinforce monitoring and evaluation (M&E) for implementing the global Roadmap and continue efforts to strengthen country level data collection and collation frameworks.

Sub-recommendations:

- i. Develop a robust M&E framework (further to recommendation 1) to assess progress on the Global Roadmap, clarifying roles and responsibilities in data collection and use; conduct periodic progress reviews and integration of lessons into re-prioritisation, and expand reporting.
- ii. Continue to expand initiatives to enhance country capacity to report on cholera; monitor and evaluate cholera responses.
- iii. Enhance and support collection of disaggregated data at the country level to further address GER concerns
- iv. Emphasise the need to facilitate more timely and transparent sharing of data by countries to focus advocacy efforts to reduce stigma of cholera (See recommendation 4).

Management response	<i>Accepted.</i> To note that actions for this recommendation are directly connected to Recommendation 1.			
Status	<i>In Progress</i>			
Key actions	<i>Responsible Entity(ies)</i>	<i>Timeline</i>	<i>Status</i>	<i>Comments</i>
Update the M&E strategy and framework based on the findings of the review of the GTFCC priorities (recommendation 1) and any subsequent refinements to the GTFCC Theory of Change and Results Framework.	WHE/EPM – GTFCC Secretariat with WHO M&E team, GTFCC SC, GTFCC TWGs, GTFCC CSP ECH/WSH	31 December 2026	In progress	Building on the work initiated by the GTFCC Secretariat in 2024, including a broad consultation and dedicated M&E technical working group, further progress was paused in early 2025 due to constraints in recruiting external support. The Roadmap M&E framework, central to assessing progress and guiding adaptations in our joint action, needs refinement.

For further information about the evaluation, please contact the WHO Evaluation Office evaluation@who.int

WHO/DGO/EVL/2025.78 - © **WHO 2026**. Some rights reserved.

In line with the WHO Evaluation Policy 2018 (EB143(9)), this publication contains an independent evaluation report by the WHO Evaluation Office. It does not reflect the views or policies of WHO.